

Telehealth Debate Speech

I want to start with a question. When you're sick — really sick — who should be traveling? You, or the doctor?

Right now, in the American healthcare system, the answer is you. You drive to the hospital. You sit in the waiting room. You navigate the parking garage, the check-in desk, the insurance verification, the billing department. You do all of this while you are the one who is sick. The doctor is already there. The hospital is already there. Everything is already there — except you. And the entire system waits for you to come to it.

Telehealth flips that. And I'm going to argue today that it should — permanently.

Let me start with what the data actually says, because this is not speculative.

Before COVID, about 1% of healthcare visits were virtual. 14% of physicians had ever used telehealth. Then the pandemic hit, and within weeks, telehealth surged to nearly 50% of all medical visits — a 766% increase. Medicare telehealth visits went from 5 million to over 53 million. And here's the thing: it worked. It didn't just survive as a stopgap. It worked.

Meta-analyses — not one study, but systematic reviews of dozens of studies — show no significant difference in outcomes between telehealth and in-person care for depression, for anxiety, for chronic disease management. A study of over 247,000 patient surveys found that satisfaction with telehealth was non-inferior to in-person care. One study found that after about five telehealth sessions, 67% of patients no longer

had clinically significant anxiety, and 62% no longer had clinically significant depression.

And it costs a fraction. Telehealth visits are five times cheaper than in-person appointments. Patients are billed \$400 less on average. CMS estimates \$60 million saved in Medicare travel costs alone. For geriatric patients, cost reductions ranged from \$223 to nearly \$4,000 per event — up to 94% savings in low-income settings.

So it works as well. It costs less. And 82% of patients say they prefer a hybrid model. 74% of physicians are still using it post-pandemic. This is not a pandemic experiment anymore. This is the new baseline.

But I don't want to just argue access and cost. Everyone argues access and cost. I want to argue architecture — because the reason telehealth matters is not just that it's cheaper. It's that it fixes something structurally broken about how we deliver care.

The hospital-centric model concentrates all of healthcare at a single node. Every patient must travel to that node, wait at that node, and submit to that node's schedule. This is the architecture of central planning. All information and all decision-making flow through one point.

And Hayek showed us, in 1945, why that fails. "The knowledge of the circumstances of which we must make use," he wrote, "never exists in concentrated or integrated form, but solely as the dispersed bits of incomplete and frequently contradictory knowledge which all the separate individuals possess." No single institution can hold the distributed knowledge of millions of patients — their schedules, their environments, their daily realities, what they actually need.

Telehealth distributes care to where the knowledge actually lives — with the patient. The provider sees the patient in their home. The encounter

happens on the patient's terms. The local knowledge of the individual informs the care. This is not just convenient. It is structurally superior.

Now, the opposition will say: "You can't do a physical exam through a screen."

They're right. So the doctor comes to you.

This is the part most people miss. The question was never telehealth versus in-person. The question is: who travels? In the current model, the patient always travels — even when they're sick, elderly, disabled, or rural. 94% of rural patients would need to travel over 70 miles for specialist care. 84% would miss a full day of work.

In the model I'm describing, telehealth handles the 90% of encounters that don't require physical presence — follow-ups, mental health, medication management, chronic disease, triage. And when a physical exam or procedure is actually needed, a mobile provider comes to the patient.

This already exists. DispatchHealth covers about 80% of what an emergency department can treat — in the patient's home. 97% patient satisfaction. Saves payers over \$1,000 per avoided ER visit and \$7,000 per avoided hospitalization.

The VA's Home-Based Primary Care program reduced hospital days by 60%, readmissions by 21%, and nursing home use by 89%. Northwell Health's house call program saved \$13,000 per patient per year.

And CMS's Hospital at Home initiative — 398 hospitals across 39 states — is treating pneumonia, heart failure, COPD exacerbations in patients' homes. The outcomes? Lower mortality than brick-and-mortar hospitals. Lower infection rates. Lower readmissions. 95% of patients rated it 9 or 10 out of 10.

The hospital didn't make the care better. The hospital introduced hospital-acquired infections, sleep disruption, delirium, and immobility. The care got better when it left the building.

And before someone says, "Well, you still need hospitals for surgery, trauma, and ICU" — let me challenge that too.

What does a hospital building actually provide? The answer used to be: heavy equipment that can't move, sterile environments that can't be replicated, specialists who need to be in the same room, and continuous monitoring that requires a ward. Every single one of those constraints has dissolved.

Surgery: In June 2025, an AdventHealth surgical team performed a robotic prostatectomy on a patient 7,000 miles away in Angola. Seven thousand miles. The surgeon was not in the same room, not in the same country, not on the same continent. Mobile surgical units carry full operating rooms on wheels — the military has operated on this model with MASH units for decades. Surgery doesn't need a building. It needs a sterile field, a skilled operator, and equipment. All three are now portable.

ICU: Tele-ICU allows remote intensivists to monitor critically ill patients via real-time video and AI-assisted alerts. Remote patient monitoring has shown a 68% reduction in ER visits and a 35% drop in hospitalizations for high-acuity patients. The monitoring doesn't need a ward. It needs sensors and a connection.

Trauma: Surgery within the golden hour at the point of injury has better survival than transport to a distant facility. Handheld ultrasound fits in a pocket. Portable CT scanners are FDA-cleared. Every minute spent transporting a patient to a centralized facility is a minute of care they are not receiving.

The hospital is not a medical necessity. It is an architectural habit inherited from an era when equipment couldn't move and information couldn't travel. Both of those constraints are gone. The building remains because the system was designed around it — not because patients need it.

Now I want to go somewhere most people don't go in this debate. I want to go to history.

Before hospitals became the center of American medicine, healthcare was already decentralized, affordable, and voluntary. And it was deliberately destroyed.

In the late 1800s and early 1900s, fraternal societies — also called mutual aid societies or lodges — were the primary source of healthcare for working-class Americans. By 1920, over one in four adult Americans were members. One in three men belonged to a mutual aid society. And they cut across every line — race, class, gender, immigration status.

Here's how lodge practice worked. A group of workers would form a lodge. They'd pool a small amount of money. They'd hire a doctor on retainer. Each member paid one to two dollars a year — about one day's wages — and got access to a doctor's care for the entire year, including basic surgery and house calls. No insurance companies. No hospital bureaucracies. No billing departments. Just a voluntary association of people and a doctor who served them directly.

And the patients controlled the arrangement. They hired the doctor, set the terms, and could replace him if he wasn't keeping people healthy. The customer had power.

This wasn't just for white men either. The Free African Society of Philadelphia, founded in 1787 by Richard Allen and Absalom Jones, provided sick benefits and community support. The Grand United Order of Odd Fellows, organized in 1843, is the oldest African American mutual aid

society. When the Knights and Daughters of Tabor opened Taborian Hospital in Mississippi in 1942 — because Black Americans were routinely denied admission to white hospitals — their membership nearly doubled to 47,000 in three years. These communities didn't wait for the government to solve their problem. They solved it themselves. Through voluntary association.

So what happened? The AMA killed it.

They called it the "lodge practice evil." They imposed sanctions on any doctor who accepted lodge contracts — denied them licenses, denied them hospital privileges, denied them professional standing. They pressured hospitals to close their doors to patients of lodge doctors. And in 1912, they gained control of medical school accreditation. They rated schools "unacceptable," and the number of medical schools in America dropped from 166 to 81. A 51% reduction. This restricted the supply of doctors, drove up prices, and made it impossible for lodge doctors to offer affordable care.

The lodge doctor who made house calls for two dollars a year was replaced by the hospital that charges two thousand dollars for an ER visit. That was not progress. That was cartelization.

And this brings me to the deepest problem with the current system. It's not cost. It's not access. It's incentives.

The modern insurance company is structurally separated from the thing it's supposed to prevent. Think about what that means. The insurer collects premiums and profits by denying claims. The less care delivered, the more money retained. We saw this in class — UnitedHealth's nH Predict algorithm doubled their denial rate. 90% of those denials were reversed on appeal. But only 0.2% of patients ever appealed. Because the denial is cheap to issue and the appeal is expensive to fight. The incentive is to deny, not to heal.

On the other side, the hospital and pharmaceutical system profits when people stay sick. Chronic disease management is recurring revenue. A cured patient is a lost customer. A managed patient is a subscription.

So the patient is trapped between two entities that both profit from their illness. The insurer profits by denying care. The provider profits by prolonging it. Nobody's incentive is health.

The fraternal society fixed this by keeping what insurance separates unified. The same community that pooled the money also hired the doctor and also lived with the consequences. If the doctor kept people healthy, the pool stayed solvent and everyone thrived. If the doctor was ineffective, the pool drained and the community suffered. The only way to win was to keep people healthy. There was no middleman extracting profit from the gap between patient and provider. The lodge was the insurance, was the patient community, was the employer of the doctor. All three functions in one voluntary body.

Telehealth restores this alignment. When a patient connects directly with a provider — without an institutional gatekeeper deciding whether the encounter is "covered," without a billing department inflating the cost, without an algorithm deciding whether to deny the claim — the relationship returns to what lodge practice was. A direct, voluntary exchange between a person who needs care and a person who provides it. The price signal is honest. The incentive is health. The middleman is gone.

And if you want the biological version of this: in a living cell, a signaling molecule connects directly to its receptor. There is no middleman molecule sitting between ligand and receptor collecting a fee. The interaction happens because the signal is present and the receptor has affinity — voluntary exchange governed by real conditions. When you insert something that corrupts that signal — an oncogene that constitutively activates a receptor regardless of actual demand — you get cancer. Growth without information. Investment without demand. Activity without purpose. The insurance company is the oncogene of healthcare.

What fraternal societies practiced was actual democracy. Not the version where you vote every four years for someone who makes decisions for you, but the version where the people affected by a decision make that decision directly, in voluntary association.

A lodge of 200 workers hiring a doctor — that is democracy. They choose who provides their care, on what terms, at what price. They can fire the doctor. They can change the terms. They have voice and exit.

Compare that to today. You "choose" an insurance plan from a handful of corporations. You have no say in which doctors are in-network, what procedures are covered, or whether a claim is approved. An algorithm you've never seen denies your care. You appeal to a bureaucracy you can't reach. You vote for politicians who promise to fix healthcare and never do. That is the facade of choice within a centralized system. The appearance of democracy without the substance.

Telehealth reopens the possibility of real choice. When a patient can connect directly with any provider — across state lines, outside institutional gatekeepers, at a transparent price — power returns to the individual. Not through voting. Through choosing.

And cells do the same thing. Every cell in a tissue reads real signals, refuses bad ones, stays alive, and works to correct the problem. Cells don't comply with broken systems. They participate in fixing them. The fraternal society operated on that principle. The modern healthcare system operates on the opposite one — you participate because you have no alternative, not because the system serves you.

So let me close with this.

The question is not whether decentralized healthcare can work. A hundred years of fraternal societies already answered that. The question is not

whether telehealth produces good outcomes. Hundreds of studies already answered that. The question is not whether hospitals are the only way to deliver complex care. Telesurgery across oceans and hospital-at-home programs with lower mortality already answered that.

The real question is whether we will let a system that profits from sickness continue to stand between patients and their care. Or whether we will do what living systems have always done — distribute the knowledge, distribute the response, and let the people closest to the problem solve it.

Telehealth is not a technology. It is a correction. And it is long overdue.

Thank you.